

24 mm

SPACE FOR PHARMACODE

For the use only of a Registered Medical Practitioner
or a Hospital or a Laboratory

SaferonTM

DROPS

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Composition:
 Each ml contains
 Iron (III) hydroxide polymaltose complex
 Equivalent to Elemental iron 50 mg
 In a syrup base.

Product Description: A printed carton containing amber coloured labeled sealed glass bottle with a dropper kept in a polythene bag containing reddish brown coloured viscous flavoured liquid with sweet taste.

Clinical Pharmacology:
Pharmacodynamics
 Iron (III) hydroxide polymaltose complex (IPC) is a combination of ferric iron with maltol (a food additive). Its non-ionic state facilitates controlled and reliable absorption and bioavailability as compared to conventional iron salts when it comes in contact with the mucosal cell surface. It is a superior molecule since it is soluble at neutral pH. The iron from polymaltose complex is available intraluminally and is not chelated by other substances. It has been shown that IPC more closely approximates the way in which red blood corpuscles handle dietary iron and thus is better tolerated by RBCs compared to conventional iron salts. Furthermore, the non-ionic state ensures that gastric irritation is not seen with IPC, a feature commonly associated with conventional iron preparations. This leads to better patient compliance, a feature not commonly seen with conventional iron preparations. These characteristics suggest an increased compliance with IPC which is crucial for effective long-term therapy with an iron preparation in IDA. The efficacy of IPC in prophylactic and therapeutic management of IDA is well documented in clinical studies.

Pharmacokinetics
 IPC has been found to be equally bioavailable for hemoglobin synthesis as ferrous salts in experimental animals as well as in human beings. The bioavailability, when given orally, is unaffected by presence of food components such as phytic acid, oxalic acid, tannin, sodium alginate, choline salts, Vitamins A, D, E, soy oil and flour unlike ferrous salts. IPC delivers the complexed iron from the serum to endogenous iron binding proteins with a half life of 90 mins, after which it is taken by liver reticuloendothelial system, as well as by transferrin, apoferritin, spleen, and bone marrow, subsequent to which it rapidly undergoes metabolism and available for erythropoiesis.

Indications:
 For prophylaxis and treatment of iron-deficiency to cover the recommended daily allowances (RDA) during childhood.

Dosage & Administration:
 Dosage and duration of therapy are dependent on the extent of iron deficiency. The daily dose can be divided into separate doses or can be taken at a time. To optimize efficacy, Saferon drops can be administered mixed in baby food or in fruit and vegetable juices.

Treatment of iron deficiency

Saferon	Manifestation of iron deficiency	Latent iron deficiency	Prophylactic therapy (RDA)
Low birth weight infants*	1-2 drops daily per kg body weight for 3-5 months	-	-
Infants (upto 1 year)	10-20 drops daily	6-10 drops daily	2-4 drops daily
Children (1-12 years)	20-40 drops daily	10-20 drops daily	4-6 drops daily

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*For low birth weight infants, the requirement is 2 mg/kg/d, to a maximum of 15 mg/d, starting at no later than two months of age. Higher doses have been suggested for in the lowest birth weight categories. Infants with birth weights between 1000 and 1500 g should receive 3 mg/kg/d and infants with birth weight less than 1000 g should receive 4 mg/kg/d. For these infants iron supplementation at higher doses should continue throughout the first year of life.

Contraindications:

1. Hypersensitivity to any of the components or other iron preparations.
2. Anaemia not caused by iron deficiency (e.g. hemolytic anaemia, disturbances in erythropoiesis, hypoplasia of the marrow)
3. Iron overload (e.g. hemochromatosis, hemosiderosis)

Warnings and Precautions:

- **Accidental overdose of iron-containing products is a leading cause of fatal poisoning in children. Keep this product out of the reach of children. In case of accidental overdose, call a doctor or poison control center immediately.**
- Iron preparations, including Iron [III] hydroxide polymaltose, can cause allergic or anaphylactoid reactions. In case of an allergic reaction, Saferon drops should be withdrawn immediately and seek medical attention.
- Do not exceed recommended dosage. The treatment of any anemic condition should be under the advice and supervision of a physician. Occasional gastrointestinal discomfort (such as nausea) may be minimized by taking with meals. Iron containing medication may occasionally cause constipation or diarrhea.
- Caution is recommended in patients with allergies and hepatic and renal insufficiency.
- Caution should be exercised in patients with gastrointestinal disease such as ulcers, colitis, etc.

Carcinogenicity, Mutagenicity and Impairment of Fertility:
No animal studies are available with Saferon drops

Drug Interactions
Iron [III] hydroxide polymaltose, like other iron preparations, may decrease the absorption of many drugs. Iron [III] hydroxide polymaltose should not be taken within 2 hours of a dose of any of the following drugs: tetracyclines, fluoroquinolones, chloramphenicol, cimetidine, levodopa, levthyroxine, methyldopa or penicillamine.

Antacids may decrease the absorption of Iron [III] hydroxide polymaltose. Antacids should not be taken within 2 hours of a dose of Iron [III] hydroxide polymaltose.

Adverse Effects
Saferon drops are well tolerated with fewer incidences of adverse events. Commonly reported adverse events include: epigastric pain, metallic taste, nausea or vomiting, abdominal discomfort, constipation, diarrhea, black stools, or temporary staining of the teeth. The non-ionic state of IPC ensures that gastric irritation is not seen with IPC, a feature commonly associated with conventional iron preparations. This leads to better patient compliance, a feature not commonly seen with conventional iron preparations.

Overdosage
No overdosage has been reported with Saferon drops. Although IPC is safer than conventional ferrous salts, overdosage may occur from accidental ingestion. Symptoms of iron overdosage include decreased energy; nausea; vomiting; abdominal pain; tarry stools; a weak, rapid pulse; fever; coma; seizures; and death. Emergency medical attention is recommended in such cases.

Storage Condition
Store below 30°C. Protect from light.

Pack Size : 15 ml

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MANUFACTURED BY:

glenmark
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